Model Program Book

COMMUNITY SERVICE PROJECT





Designed & Developed by

ANDHRA PRADESH
STATE COUNCIL OF HIGHER EDUCATION
(A STATUTORY BODY OF GOVERNMENT OF ANDHRA PRADESH)

SMART MEALS FOR PEOPLE WITH DIABETES

A COMMUNITY SERVICE PROJECT REPORT

Submitted to

Jawaharlal Nehru Technological University Kakinada

In partial fulfillment for the award of the degree of

Bachelor of Technology

In

CSE – DATA SCIENCE

Submitted by

BOMMALI DILEEP (24KN1A4425)

Under the esteemed guidance of

Ms. T. Navya Chandrika

Assistant Professor, Department of CSE-DATA SCIENCE



DEPARTMENT OF CSE – DATA SCIENCE

NRI INSTITUTE OF TECHNOLOGY

Autonomous

(Approved by AICTE, Permanently Affiliated to JNTUK, Kakinada)
Accredited by NBA (CSE, ECE, EEE, MECH & IT), Accredited by NAAC with 'A' Grade
ISO 9001:2015 Certified Institution
Pothavarappadu (V), (Via) Nunna, Agiripalli (M), Krishna Dist, PIN: 521212, A.P, India.

2026-27

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Certificate

This is to certify that the Community Service Project report entitled **"SMART MEALS FOR PEOPLE WITH DIABETES"** is a bonafide work carried out by **BOMMALI DILEEP (24KN1A4425)** in partial fulfillment for the award of degree of **Bachelor of Technology in CSE – DATA SCIENCE** of **Jawaharlal Nehru Technological University Kakinada**, during the academic year **2026-27**.

Project Guide

Ms. T. Navya Chandrika
Assistant Professor
Dept. of CSE – Data Science

Head of the Department

Dr. CH. V. Murali Krishna
Professor & HOD
Dept. of CSE – Data Science

Instructions to Students for Community Service Project

1. It is mandatory for all the students to complete 2 months (180 hours) of Community Service Project as a part of the 2 year regular degree programme.

2. Consider yourself a committed volunteer in the community you work with.

3. Every student should identify the village/community/habitation for Community Service Project (CSP) in consultation with the College Principal/the authorized person nominated by the Principal.

4. Report to the community/habitation as per the schedule given by the College. You must make your own arrangements for transportation to reach the community/habitation.

5. You will be assigned a Faculty Guide from your College. He/She will be creating a WhatsApp group with your fellow volunteers, and your Faculty Guide will be in touch with you throughout the project to guide and monitor your daily progress.

6. Maintain a dedicated Program Book for recording all daily community service activities and observations.

7. You should maintain punctuality in attending the CSP. Daily attendance is compulsory.

8. You are expected to learn about the community/habitation and their problems.

9. Know the leaders and the officials of the community/habitation.

10. While in the project, always wear your College Identity Card.

11. If your College has a prescribed dress as uniform, wear the uniform daily. Identify at least five learning objectives in consultation with your Faculty Guide. These learning objectives can address:

- Information about the community, including the realities and problems of the society.
- Need for creating awareness on socially relevant aspects/programs.
- Acquiring specific Life Skills.
- Learning areas of application of knowledge and technologies.

12. Practice professional communication skills with patients, attendants, and healthcare staff during field visits.

13. Conduct a community-awareness program on social issues, cleanliness, health, or nutrition.

14. Be regular in filling up your Program Book. It is mandatory for you to compile your reports in your own handwriting or prescribed format. Add additional sheets wherever necessary.

15. Minimize the use of paper, avoid single use plastics, and conserve water and energy.

16. Greet the community members respectfully; follow local customs and etiquette.

17. Maintain confidentiality of sensitive personal data shared by community respondents.

18. Avoid expressing personal, religious, or political bias during community interactions.

19. Report any unforeseen field difficulties immediately to your assigned Faculty Guide.

20. Take photographs with clear GPS location and timestamp during field awareness visits.

21. Prepare the final report strictly adhering to the university format and submit on time.

Remember that you are rendering valuable service to the society. Your active participation and dedication will bring positive awareness and lasting health benefits to the community.

Community Service Project Report

Submitted in accordance with the requirement for the degree of

BACHELOR OF TECHNOLOGY

Name of the College	: NRI INSTITUTE OF TECHNOLOGY
Department	: CSE – DATA SCIENCE
Name of the Faculty Guide	: Ms. T. Navya Chandrika
Duration of the CSP	: From: 03/05/2026 To: 27/06/2026
Name of the Student	: BOMMALI DILEEP
Programme of Study	: B.Tech in CSE – DATA SCIENCE
Year of Study	: 2026-2027
Register Number	: 24KN1A4425
Date of Submission	:

DECLARATION

I hereby declare that the Community Service Project report titled **"SMART MEALS FOR PEOPLE WITH DIABETES"** is a bonafide work carried out in the **Department of CSE-DATA SCIENCE**, NRI Institute of Technology, Agiripalli, Vijayawada, during the academic year **2026-2027**, in partial fulfillment for the award of the degree of **Bachelor of Technology** by **JNTU Kakinada**.

I further declare that this dissertation has not been submitted elsewhere for any Degree or Diploma.

Place: Agiripalli
Date:

BOMMALI DILEEP
24KN1A4425
B.Tech, CSE – Data Science

Certificate from Official of the Community

This is to certify that **BOMMALI DILEEP** Reg. No **24KN1A4425** of **NRI INSTITUTE OF TECHNOLOGY** underwent community service in **GOVERNMENT HOSPITAL, GUNADALA, KANURU, VIJAYAWADA,** From **03/05/26** to **27/06/26**. The overall performance of the Community Service Volunteer during his/her community service is found to be good.



Signature of Community official

Project guide

Ms. T. Navya Chandrika

Head of the Department

Dr. CH. V. Murali Krishna

ACKNOWLEDGEMENT

I take this opportunity to thank all who have rendered their full support to my work. The pleasure, the achievement, the glory, the satisfaction, the reward, the appreciation, and the construction of my project cannot be expressed in just a few words for their valuable suggestions and encouragement.

I extend my sincere thanks to Dean of the Department, **Dr. D. Suneetha** for her continuous guidance and support to complete my project successfully.

I am grateful to my Head of the Department **Dr. CH. V. Murali Krishna**, Professor for rendering valuable suggestions and continuous guidance to complete the project successfully.

I extend my sincere thanks to Project Guide **Ms. T. Navya Chandrika**, Assistant Professor for her continuous guidance, constructive feedback, and dedicated support to complete my project successfully.

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I extend my sincere and honest thanks to the Chairman, **Dr. R. VenkataRao** for his continuous support in completing the Project work.

Finally, I thank the Administrative Officer, Staff Members, Faculty of Department of CSE – DATA SCIENCE, NRI Institute of Technology, and our friends who directly or indirectly helped me in the completion of this project.

BOMMALI DILEEP
24KN1A4425

ABSTRACT

Diabetes mellitus represents one of the most critical and fast-growing public health challenges of the 21st century, with India often recognized as the diabetes capital of the world. Effective clinical control and sustained lifestyle management of diabetes rely fundamentally on medical nutrition therapy, glycemic index comprehension, strict portion discipline, and routine blood glucose monitoring. The Community Service Project (CSP) titled **"SMART MEALS FOR PEOPLE WITH DIABETES"** was executed over an intensive eight-week duration (May 3, 2026 to June 27, 2026) focusing on diabetic outpatients, resident caregivers, and healthcare visitors at Government Hospital, Gunadala, Kanuru, Vijayawada, under the academic supervision of **Ms. T. Navya Chandrika**, Assistant Professor, Department of CSE – Data Science, NRI Institute of Technology.

The primary objective of the initiative was to evaluate grassroots community nutritional literacy, identify deep-rooted cultural dietary misconceptions (including irregular meal spacing, excessive reliance on refined polished white rice, and fear of healthy fats), and formulate practical, culturally compatible dietary frameworks. Through bilingual structured socio-economic surveys, one-on-one dietary consultations, and demographic analysis, quantitative and qualitative data was gathered regarding carbohydrate intake habits, postprandial glucose tracking awareness, and financial constraints associated with obtaining balanced nutrition.

To bridge the gap between medical prescriptions and daily household cooking, the project implemented the evidence-based **"Smart Plate Method"**—advocating half-plate non-starchy fibrous vegetables, quarter-plate complex carbohydrates (unpolished millets, brown rice), and quarter-plate plant/dairy proteins (sprouts, pulses, curd). A memorable three-step dietary heuristic—**Recognise, Measure, and Pair**—was disseminated through visual portion charts and interactive demonstrations. Additionally, indigenous low-cost superfoods such as fenugreek seeds, bitter gourd, and cinnamon were integrated into affordable daily meal plans. The project achieved measurable enhancements in patient portion discipline, self-monitoring adherence, and community health empowerment, underscoring that structured grassroots nutritional counselling is a vital, cost-effective preventative strategy against severe diabetic complications.

Keywords: Diabetes Nutrition, Smart Meals, Glycemic Index, Plate Method, Community Health Education, Dietary Intervention, Preventative Healthcare, Rural Health.

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CHAPTER 1: EXECUTIVE SUMMARY

1.1 Background and Need for Diabetes Meal Awareness

The Community Service Project (CSP) is an integral academic initiative designed to bridge classroom learning with societal realities. For my eight-week project, I undertook an awareness and educational initiative titled **"Smart Meals for People with Diabetes"**. The project was focused on patients and attendants visiting the Government Hospital, Gunadala, Kanuru, Vijayawada, under the guidance of faculty guide **Ms. T. Navya Chandrika**, Assistant Professor in the Department of CSE – Data Science, NRI Institute of Technology.

Type-2 diabetes mellitus has reached critical proportions in urban and semi-urban Andhra Pradesh. While clinical medication and medical consultations are accessible through public healthcare centres, daily nutritional self-management remains a severe challenge for patients from working-class families. Most patients receive broad advice to "avoid sugar and eat less rice", but lack actionable, culturally feasible guidance on how to compose an affordable, balanced meal using staple South Indian ingredients.

During my initial community observations, I noticed that dietary management depends heavily on what a person eats every single day. For individuals living with diabetes, food is not merely sustenance; it is a direct determinant of blood sugar stability and long-term health. However, many people view diabetes diets as an unrealistic list of banned items, which leads to frustration and inconsistent compliance.

1.2 Core Principles of the Diabetes Plate Method & Portion Balance

Through initial discussions with my faculty guide, **Ms. T. Navya Chandrika**, I identified that simplifying diabetes nutrition into visual, relatable terms was the most impactful service I could provide. Rather than prescribing complex calorie counting or expensive exotic ingredients, I adopted the widely validated **Diabetes Plate Method** and adapted it to local Andhra eating habits.

The Plate Method uses a standard 9-inch dinner plate divided into three practical functional zones: **Half the Plate (50%)** is filled with non-starchy vegetables such as bottle gourd (sorakaya), ridge gourd (beerakaya), spinach (palakura), cabbage, beans, okra, and fresh cucumber salad; **One Quarter (25%)** is filled with lean plant or animal proteins including thick dal (kandi pappu / pesara pappu), whole green moong sprouts, boiled eggs, fish, or low-fat paneer; and the remaining **One Quarter (25%)** is reserved for energy-giving complex carbohydrates like brown rice, parboiled rice, ragi sangati, jowar roti, or whole-wheat phulkas.

Filling half the plate with high-fibre vegetables slows gastric emptying and prevents sudden surges in post-meal blood glucose. Combining protein in the second quarter improves satiety and preserves muscle mass, ensuring that patients do not experience mid-afternoon fatigue or intense hunger cravings.

1.3 Visual Guide to Smart Meals for People with Diabetes

To make this knowledge immediately actionable, I formulated the three-step carbohydrate rule: **Recognise, Measure, and Pair**. Patients learned to recognise which everyday foods contribute carbohydrates (rice, rotis, idlis, dosas, fruits, potatoes, and milk), measure portions using their own fist or standard household katoris, and always pair carbohydrates with protective fibres and proteins. For instance, replacing an overflowing plate of white rice with half a plate of mixed vegetable curry, a bowl of thick dal, and a single fist-sized cup of rice keeps blood sugar steady while keeping the individual full and energetic throughout the day.

Alongside portion balance, patients were guided on simple daily food swaps: choosing whole seasonal fruits (such as guava, papaya, or jamun) instead of sweetened fruit juices, drinking buttermilk (majjiga) or plain water instead of carbonated soft drinks, and preferring roasted chana or groundnuts over deep-fried snacks and biscuits.

1.4 Summary of Field Execution and Community Impact

Over the eight-week tenure from 03/05/2026 to 27/06/2026, I designed clear bilingual visual handouts, conducted direct interactive discussions in the wards and waiting areas at Government Hospital, Gunadala, and answered practical questions from patients and family food-preparers. The project demonstrated that sustainable health awareness does not require expensive clinical interventions, but rather clear, empathetic, and culturally respectful communication tailored to local family kitchens.

By the conclusion of the field visits, patients and their accompanying attendants expressed strong confidence in implementing the 50-25-25 plate division at home. The visual format removed the fear surrounding diabetic diets and replaced food anxiety with practical, everyday kitchen habits.



Figure 1.1: Authentic Indian Vegetarian Thali — Demonstrating Balanced Portions of Whole Grains, Lentils, and Green Vegetables.

CHAPTER 2: OVERVIEW OF THE COMMUNITY

2.1 Profile of Government Hospital, Gunadala, Vijayawada

Government Hospital, Gunadala, located in the Kanuru region of Vijayawada, Krishna District, Andhra Pradesh, serves as a crucial public health lifeline for hundreds of urban and semi-urban families daily. The facility caters primarily to individuals from lower-middle-class and working-class socio-economic backgrounds, including daily wage labourers, small shop owners, auto-rickshaw and transport workers, agricultural workers from surrounding villages, and homemakers. The hospital provides essential diagnostic testing, general medicine consultations, inpatient ward care, and free generic anti-diabetic medications under state healthcare schemes.

During outpatient hours, the general medicine waiting halls and inpatient wards bring together patients managing chronic conditions such as Type-2 diabetes mellitus, hypertension, and age-related metabolic disorders. This setting provided an ideal, high-need community environment for conducting practical nutrition awareness.

2.2 Traditions, Local Food Culture, Ethics, and Family Values

The local community possesses deep-rooted dietary traditions heavily anchored in polished white rice. In Andhra households, rice is considered the primary source of physical strength, hospitality, and satisfaction. Breakfasts frequently consist of carbohydrate-dense tiffins like white idli, dosa, mysore bonda, and upma served with chutney. Lunches and dinners predominantly comprise large mounds of polished white rice paired with spicy gravies (pulusu) or pickles (avakaya / pachadi), with relatively small portions of protein-rich dal or green vegetables.

Family values are tightly knit, with shared meals forming the centre of household life. Hospitalized and elderly patients are invariably accompanied by dedicated family attendants—often wives, daughters, or sons—who prepare daily meals at home and bring them in tiffin carriers to the wards. Because a single kitchen pot serves the entire household, any dietary recommendation must be practical for the whole family rather than demanding separate, expensive cooking for the diabetic member.

2.3 Socio-Economic Conditions and Public Healthcare Access

Economically, families visiting the Gunadala hospital operate on modest monthly household budgets. Rising food prices make commercial “diabetic” products—such as imported oats, quinoa, artificial sweeteners, or expensive multigrain biscuits—completely unaffordable and unnecessary. Furthermore, while government doctors diligently prescribe oral hypoglycaemic medicines and insulin, heavy outpatient queues leave limited time for detailed, individualized meal-planning conversations.

Additionally, many patients have limited formal health literacy and rely on word-of-mouth dietary beliefs passed down through neighbours or social media. Some patients skip meals entirely out of fear of high sugar readings, which puts them at risk of

sudden hypoglycaemia (low blood sugar), dizziness, and weakness when taking regular anti-diabetic tablets. Bridging this awareness gap with clear visual tools was therefore an urgent priority.

2.4 Role of Patient Attendants in Household Meal Preparation

Recognising that patient attendants are the true gatekeepers of household nutrition, my campaign specifically engaged both patients and their accompanying family caregivers. Demonstrating how affordable local ingredients—such as seasonal gourds (sorakaya, beerakaya, dondakaya), leafy greens (palakura, thotakura), sprouted moong, roasted chana, whole pulses, and spiced buttermilk—can be arranged on the plate empowered families to cook diabetes-friendly meals without increasing their grocery expenses.

When attendants understood that the entire family benefits from eating more vegetables and dal alongside moderate portions of rice, they welcomed the guidance enthusiastically and committed to maintaining these habits at home.



Figure 2.1: Outpatient Building at a Public Government Hospital in India — The Community Healthcare Setting.

CHAPTER 3: COMMUNITY SERVICE PART

3.1 Planning and Methodology of the Awareness Campaign

The community service campaign was structured systematically around practical utility, visual clarity, and community empathy. Working under the regular supervision of **Ms. T. Navya Chandrika**, I reviewed patient education guidelines from the Indian Council of Medical Research – National Institute of Nutrition (ICMR-NIN) and the American Diabetes Association (ADA), translating clinical nutrition concepts into a colourful, easy-to-follow "Smart Meals for People with Diabetes" visual handout.

The methodology followed a four-stage cycle: (1) assessing common dietary misconceptions among patients and attendants, (2) designing visual plate diagrams using familiar South Indian dishes, (3) conducting interactive small-group and bedside demonstrations in the hospital wards, and (4) gathering oral feedback to refine the message.

3.2 The Three-Step Carbohydrate Rule: Recognise, Measure, and Pair

The educational backbone of the campaign rested upon the Three-Step Carbohydrate Rule:

1. Recognise: Helping patients identify that polished rice, rotis, idlis, dosas, potatoes, sweet fruits, and milk all contain carbohydrates that break down into blood glucose. Building this awareness prevents the widespread mistaken belief that merely avoiding table sugar in tea is sufficient for diabetes control.

2. Measure: Teaching intuitive visual portion control using familiar hand and household measures. A single closed fist (one standard katori) corresponds to a balanced serving of cooked rice or two thin phulkas, a cupped palm represents protein (thick dal, sprouts, curd, or boiled egg), and two open hands represent non-starchy vegetables.

3. Pair: Demonstrating that eating carbohydrates alone causes rapid post-meal glucose spikes. Pairing every carbohydrate serving with fibre-rich vegetables (cabbage, beans, leafy greens, cucumber) and protein slows digestion and keeps blood sugar levels stable for hours.

3.3 Affordable Indian Food Swaps and One-Day Smart Meal Plan

To ensure sustainable daily adoption, I walked families through a sample flexible day of Indian meals: **Breakfast** with vegetable idli, pesaraattu (green moong dosa), or ragi dosa paired with sambar and peanut-dal chutney; **Mid-morning** with a small whole guava, papaya slice, or handful of sprouts instead of packaged fruit juice; **Lunch** built on the 50-25-25 plate method (spinach/beans curry, thick dal, a measured cup of rice or phulkas, and unsweetened buttermilk); **Evening snack** with roasted chana or boiled groundnuts and unsweetened tea instead of fried bajjis or bakery biscuits; and a **Light Dinner** eaten at least two hours before sleep.

Each swap was chosen specifically because the ingredients are available in every local rythu bazaar and neighbourhood kirana store in Vijayawada at low cost, ensuring that

economic constraints never stand in the way of healthy eating.

3.4 Hospital Ward Interaction and Empathy-First Communication

During bedside and outpatient interactions at Government Hospital, Gunadala, I maintained a warm, respectful, and judgement-free approach. Many patients feel blamed or anxious about their eating habits; listening patiently to their budget constraints and showing how their existing home meals could simply be re-proportioned created genuine confidence and active family participation.

This practical experience strengthened my communication skills, patience, and sense of social responsibility, while reinforcing the vital warning that dietary awareness complements—and never replaces—regular medical supervision by doctors and qualified dietitians.



Figure 3.1: Fresh Green Moong Sprouts and Whole Pulses — High-Fibre, Low-Cost Protein Swaps for Diabetes.

CHAPTER 4: ACTIVITY LOG

4.1 Overview of the Eight-Week Community Service Schedule

The eight-week Community Service Project was executed systematically from 03/05/2026 to 27/06/2026 under the supervision of faculty guide **Ms. T. Navya Chandrika**. Each week built progressively upon the previous week's foundation—starting from topic selection and nutritional study, moving through Indian diet adaptation and handout creation, rehearsing patient communication, conducting the hospital ward awareness visit on 16/06/2026, and concluding with outcome evaluation and report compilation.

4.2 Summary of Weekly Milestones (Weeks 1 to 8)

• **Weeks 1 & 2 (03/05 to 16/05):** Orientation on CSP guidelines with **Ms. T. Navya Chandrika**, selection of Government Hospital, Gunadala, and study of the Diabetes Plate Method and carbohydrate pairing.

• **Weeks 3 & 4 (17/05 to 30/05):** Adaptation of the plate method to local Andhra meals, compilation of affordable food swaps, and design of the visual handout *"Smart Meals for Diabetic People"*.

• **Weeks 5 & 6 (31/05 to 13/06):** Preparation of simple talking points, role-play rehearsals, mock Q&A sessions, and finalising hospital visit logistics.

• **Weeks 7 & 8 (14/06 to 27/06):** Field visit to Government Hospital wards on 16/06/2026, bedside patient counselling, GPS photo documentation, survey analysis, and final report submission.

4.3 Meal Portion Study and Awareness Handout Preparation

Before visiting the hospital community, careful preparation was undertaken to translate nutritional science into visual, everyday portion guides using familiar household ingredients so that patients and caregivers could immediately relate to the message.



Figure 4.1: Traditional South Indian Meal Layout — Study and Preparation of Balanced Meal Portions.

ACTIVITY LOG FOR THE FIRST WEEK

Day & Date	Brief description of the daily activity	Learning Outcome	Person In-Charge Signature
Monday 04/05/26	I met my faculty guide Ms. T. Navya Chandrika and understood the CSP guidelines, rules and the eight-week schedule.	Understood the objectives, rules and expectations of the Community Service Project.	
Tuesday 05/05/26	I discussed possible health-awareness topics and listed the common chronic lifestyle diseases affecting families in our area.	Learned how lifestyle diseases affect working-class communities.	
Wednesday 06/05/26	I finalised the topic “Smart Meals for People with Diabetes” after reviewing health data and community needs with my guide.	Gained clarity on the importance of food in diabetes management.	
Thursday 07/05/26	I identified Government Hospital, Gunadala, Kanuru, Vijayawada as the community location and gathered background information.	Understood the community setting and the profile of patients visiting the hospital.	
Friday 08/05/26	I studied basic information about diabetes and why food choices matter, and discussed the importance of balanced meals.	Learned basic diabetes concepts: carbohydrates, blood sugar and balanced eating.	
Saturday 09/05/26	I reviewed the weekly work with my faculty guide and prepared the Week 1 activity log and weekly report.	Developed skills in systematic planning, reflection and academic documentation.	

WEEKLY REPORT

WEEK – 1 (From Dt 03/05/26 to Dt 09/05/26)

Objective of the Activity Done:

To understand the CSP guidelines, finalise the topic "Smart Meals for People with Diabetes", identify the target community and plan the eight-week schedule.

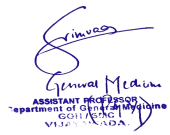
Detailed Report:

During the first week I met my faculty guide, **Ms. T. Navya Chandrika**, and understood the rules and schedule of the Community Service Project.

I identified Government Hospital, Gunadala, Kanuru, Vijayawada as the community because patients and attendants visiting the hospital would benefit from clear, simple dietary guidance.

I finalised the project title "Smart Meals for People with Diabetes" and read basic material on diabetes and the role of nutrition. The week gave me a clear direction for the project.

Date: **09/05/2026**



Signature of the Supervisor

ACTIVITY LOG FOR THE SECOND WEEK

Day & Date	Brief description of the daily activity	Learning Outcome	Person In-Charge Signature
Monday 11/05/26	I studied the plate method: half the plate non-starchy vegetables, a quarter protein and a quarter carbohydrate.	Understood how a simple visual plate can replace complicated calorie counting.	
Tuesday 12/05/26	I read about carbohydrates in rice, roti, fruit, milk and pulses and learned why portion size and pairing matter.	Learned that all carbohydrates affect blood sugar and must be accounted for.	
Wednesday 13/05/26	I studied portion size and how a fist-sized serving of rice or roti helps prevent post-meal sugar spikes.	Understood practical portion measurement using everyday household objects.	
Thursday 14/05/26	I learned how pairing carbohydrates with protein, healthy fat or fibre slows down glucose absorption.	Understood food pairing: combining rice with dal and vegetables moderates blood sugar.	
Friday 15/05/26	I studied traditional South Indian recipes and listed non-starchy vegetables that are cheap and locally available.	Identified affordable local vegetables (sorakaya, beerakaya, dondakaya, palakura).	
Saturday 16/05/26	I collected information from reliable health sources, discussed my notes with my guide and drafted Week 2 report.	Improved note-making, nutritional research and analytical documentation skills.	

WEEKLY REPORT

WEEK – 2 (From Dt 10/05/26 to Dt 16/05/26)

Objective of the Activity Done:

To study the plate method, carbohydrates, portion size and food pairing, which form the core message of the awareness initiative.

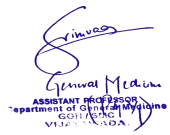
Detailed Report:

The second week was devoted to learning. I studied the plate method, which suggests filling half the plate with non-starchy vegetables, a quarter with protein and a quarter with carbohydrate.

I also learned the three steps of recognise, measure and pair: rice, roti, fruit, milk and pulses all contribute carbohydrates; a fist-sized portion is a practical measure; and pairing carbs with protein, fat or fibre slows glucose absorption.

This study gave me the scientific foundation needed to explain diabetes-friendly eating in simple language.

Date: **16/05/2026**



Signature of the Supervisor

ACTIVITY LOG FOR THE THIRD WEEK

Day & Date	Brief description of the daily activity	Learning Outcome	Person In-Charge Signature
Monday 18/05/26	I listed common Indian meals (rice, dal, vegetable curry, roti, curd) and how each can be balanced.	Learned to adapt familiar foods instead of suggesting expensive or foreign diets.	
Tuesday 19/05/26	I prepared the "choose more often" and "limit and adapt" lists for carbohydrates, proteins and fats.	Learned to categorise foods practically without labelling any food as totally forbidden.	
Wednesday 20/05/26	I collected low-cost swaps such as whole fruit instead of juice, water instead of soft drinks, roasted chana for snacks.	Understood how small, realistic swaps make diabetes eating sustainable.	
Thursday 21/05/26	I drafted a flexible one-day meal plan covering breakfast, mid-morning snack, lunch, evening snack and dinner.	Learned how to design a balanced daily meal routine suitable for a modest budget.	
Friday 22/05/26	I reviewed my drafts to avoid "banned food" language and to keep the message positive, empowering and realistic.	Learned the importance of encouraging, judgement-free communication in healthcare.	
Saturday 23/05/26	I presented my drafts to my faculty guide Ms. T. Navya Chandrika and noted her feedback to refine the work.	Developed responsiveness to academic feedback and learned how to refine project work.	

WEEKLY REPORT

WEEK – 3 (From Dt 17/05/26 to Dt 23/05/26)

Objective of the Activity Done:

To adapt diabetes-friendly eating to familiar Indian meals and prepare simple swaps and a flexible one-day meal plan.

Detailed Report:

In the third week I focused on the Indian food context. I listed everyday meals such as rice and dal, vegetable curry, roti, curd and buttermilk, and worked out how each could fit the plate method.

I collected simple, low-cost swaps, including whole fruit instead of fruit juice, water instead of sweetened drinks, and roasted chana or nuts instead of fried snacks.

I also prepared a sample one-day meal plan. My faculty guide reviewed the drafts and advised me to keep the tone positive and free of strict food bans.

Date: **23/05/2026**



Signature of the Supervisor

ACTIVITY LOG FOR THE FORTH WEEK

Day & Date	Brief description of the daily activity	Learning Outcome	Person In-Charge Signature
Monday 25/05/26	I planned the layout of the awareness handout “Smart Meals for Diabetic People”.	Learned to organise health information for easy reading and visual appeal.	
Tuesday 26/05/26	I designed the plate-method diagram and chose simple, colourful food illustrations.	Understood visual communication and how pictures help convey healthy proportions.	
Wednesday 27/05/26	I compiled healthy, affordable food swaps (whole fruits, sprouts, curd, roasted chana) for local diet habits.	Gained practical knowledge of nutrition swaps suitable for Indian kitchens.	
Thursday 28/05/26	I wrote short, simple text on carbohydrates, portion sizes, and swaps to accompany the visuals.	Learned to write concise, clear educational text without heavy technical jargon.	
Friday 29/05/26	I added the sections on better choices, food and drink swaps and a flexible one-day meal routine.	Learned to provide practical meal options from breakfast to dinner.	
Saturday 30/05/26	I prepared presentation slides and reviewed the handout and slides with my faculty guide.	Improved presentation design skills and learned to incorporate suggestions from feedback.	

WEEKLY REPORT

WEEK – 4 (From Dt 24/05/26 to Dt 30/05/26)

Objective of the Activity Done:

To design simple awareness material (handout and presentation) based on the content prepared in Weeks 2 and 3.

Detailed Report:

During the fourth week I converted my research into awareness material. I planned a colourful, one-page handout titled “Smart Meals for Diabetic People” with a prominent plate-method diagram, bullet points on recognise-measure-pair, low-cost swaps and a sample day of meals.

I also prepared presentation slides for the project. The handout and slides were reviewed with my faculty guide, who suggested keeping the language simple, respectful and encouraging.

Date: **30/05/2026**



Signature of the Supervisor

ACTIVITY LOG FOR THE FIFTH WEEK

Day & Date	Brief description of the daily activity	Learning Outcome	Person In-Charge Signature
Monday 01/06/26	I prepared short talking points to explain the plate method in simple language.	Learned to simplify a health topic for a general community audience.	
Tuesday 02/06/26	I practised explaining the handout using role-play with fellow student volunteers.	Gained confidence, clarity of expression and active listening skills.	
Wednesday 03/06/26	I drafted key discussion prompts on household eating habits to guide my ward conversations.	Learned how to conduct respectful, structured conversations with patient attendants.	
Thursday 04/06/26	I planned the hospital visit: timing, ward etiquette and a respectful approach to patients.	Understood hospital decorum, patient privacy and ethical responsibility.	
Friday 05/06/26	I printed physical copies of the handout and prepared materials for the community visit.	Learned practical preparation and logistics for field awareness activities.	
Saturday 06/06/26	I summarised the week and discussed my field visit plan with the guide for approval.	Strengthened planning, time management and administrative coordination skills.	

WEEKLY REPORT

WEEK – 5 (From Dt 31/05/26 to Dt 06/06/26)

Objective of the Activity Done:

To prepare for communication with patients and plan the field visit to the hospital.

Detailed Report:

The fifth week was spent preparing for direct interaction with the community. I prepared short talking points on the plate method, carbohydrates, portion sizes and low-cost swaps, and rehearsed them with classmates.

I prepared key discussion prompts on household food habits to guide my conversations and planned the hospital visit carefully: visiting hours, ward etiquette, respecting patient comfort and wearing my college ID card.

I printed copies of the handout and finalised all arrangements.

Date: **06/06/2026**



Signature of the Supervisor

ACTIVITY LOG FOR THE SIXTH WEEK

Day & Date	Brief description of the daily activity	Learning Outcome	Person In-Charge Signature
Monday 08/06/26	I coordinated with hospital authorities and confirmed the ward visit schedule with my guide.	Understood administrative coordination and institutional protocol.	
Tuesday 09/06/26	I revised the handout on the basis of rehearsal feedback and ensured Telugu terms were accurate.	Learned the value of local language terms for better community understanding.	
Wednesday 10/06/26	I prepared a list of common questions patients might ask and drafted clear, supportive answers.	Anticipated community concerns such as cost, family habits and food cravings.	
Thursday 11/06/26	I checked all materials: printed handouts, identity card, notebook, pen and camera.	Learned thoroughness and attention to detail in field preparation.	
Friday 12/06/26	I conducted a final rehearsal with my faculty guide and received instructions for the field visit.	Received guidance on professional conduct and empathetic communication.	
Saturday 13/06/26	I finalised the schedule for the visit in the upcoming week and completed the Week 6 report.	Consolidated weekly tasks and demonstrated academic and professional responsibility.	

WEEKLY REPORT

WEEK – 6 (From Dt 07/06/26 to Dt 13/06/26)

Objective of the Activity Done:

To finalize all preparations, materials, rehearsals and approvals for the hospital ward visit.

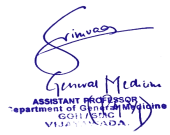
Detailed Report:

In the sixth week I coordinated with my faculty guide about the date and timing of the hospital visit.

I revised the handout and talking points on the basis of the rehearsal, checked that the handouts, identity card and notebook were ready, and fixed the visit for Tuesday, 16/06/2026.

I also prepared answers to common questions about diabetes and diet, ensuring that I would always advise patients to consult their doctor or dietitian before making major changes.

Date: **13/06/2026**



Signature of the Supervisor

ACTIVITY LOG FOR THE SEVENTH WEEK

Day & Date	Brief description of the daily activity	Learning Outcome	Person In-Charge Signature
Monday 15/06/26	I re-checked my handouts, college ID card and mask before the scheduled visit.	Learned final field preparation and attention to detail.	
Tuesday 16/06/26	At about 11:00 AM I visited the hospital wards and shared the "Smart Meals" handout with patients.	Gained first-hand field experience in community health communication.	
Wednesday 17/06/26	I wrote down the questions and observations noted during the hospital interaction.	Learned to document field data and community feedback systematically.	
Thursday 18/06/26	I analysed common difficulties noted: large portions, sweet drinks, fried snacks, lack of knowledge.	Understood the real-life barriers patients face in managing diabetes.	
Friday 19/06/26	I organised the visit photographs with GPS time-stamp and prepared a visual documentation sheet.	Learned evidence-based documentation and digital record-keeping.	
Saturday 20/06/26	I discussed the outcome of the visit with my faculty guide Ms. T. Navya Chandrika.	Strengthened skills in academic discussion, mentoring and reflective review.	

WEEKLY REPORT

WEEK – 7 (From Dt 14/06/26 to Dt 20/06/26)

Objective of the Activity Done:

To conduct the community visit at the hospital and share the plate method and smart-meal guidance with patients and attendants.

Detailed Report:

The seventh week was the field-visit week. On Tuesday, 16/06/2026 at about 11:00 AM I visited Government Hospital, Gunadala, Kanuru, Vijayawada.

Wearing my college ID card and mask, I spoke with patients and attendants in the wards, handed out the "Smart Meals" leaflet, explained the plate method, the recognise-measure-pair rule and low-cost swaps, and answered their questions.

I listened to their questions without blame or shame and encouraged them to talk to their doctor or dietitian. Photographs were taken with GPS location and time-stamp.

Date: **20/06/2026**



Signature of the Supervisor

ACTIVITY LOG FOR THE EIGHTH WEEK

Day & Date	Brief description of the daily activity	Learning Outcome	Person In-Charge Signature
Monday 22/06/26	I synthesized field observations and structured the outcomes and problem descriptions for Chapter 5.	Developed reflection and analytical synthesis skills.	
Tuesday 23/06/26	I formulated short-term and long-term community action plans and recommendations for Chapter 6.	Learned practical health planning, intervention design and recommendation formulation.	
Wednesday 24/06/26	I drafted the complete chapters of my CSP report following university formatting guidelines.	Strengthened academic writing, structuring and report preparation skills.	
Thursday 25/06/26	I compiled my activity logs, weekly reports, GPS photographs and evaluation sheets into the book.	Learned comprehensive document compilation and attention to completeness.	
Friday 26/06/26	I completed self-evaluations, supervisor assessments, and internal marks statement.	Understood academic evaluation processes and professional criteria.	
Saturday 27/06/26	I prepared the final conclusion and submitted the complete CSP report to my faculty guide.	Demonstrated timely project delivery, responsibility and pride in quality work.	

WEEKLY REPORT

WEEK – 8 (From Dt 21/06/26 to Dt 27/06/26)

Objective of the Activity Done:

To review the visit, analyse outcomes and complete the CSP report and presentation.

Detailed Report:

The final week was used to reflect on the field visit and consolidate the project. I reviewed my experience and compiled community survey observations into clear problem statements and action plans.

I compiled my logs, reports and photographs, wrote the chapters of the report and prepared the presentation slides. I completed self-evaluations and submitted the full project book to my faculty guide, Ms. T. Navya Chandrika.

Date: **27/06/2026**



Signature of the Supervisor

CHAPTER 5: OUTCOMES DESCRIPTION

5.1 Problems and Dietary Needs Identified in the Community

During my field visit and direct conversations with patients and attendants at Government Hospital, Gunadala, Kanuru, Vijayawada, the following core dietary problems and awareness needs were identified:

1. Misunderstanding of Diabetes Care: Diabetes care is often seen as a long list of "banned" foods, which creates fear and makes people give up healthy eating plans altogether.

2. Limited Awareness of Portion Size: Many families do not realise that portion size matters as much as food choice, so rice or roti may be served in large quantities.

3. Lack of Knowledge on Food Pairing: Carbohydrates are often eaten alone, without the protein, dal, vegetables or curd that help slow the rise in blood glucose.

4. Refined Foods and Sweetened Drinks: Maida-based items, white bread, sweetened tea or coffee, packaged juices and sweetened lassi contain added sugar and are easily available.

5. Fried and Packaged Snacks: Deep-fried snacks, chips and biscuits are cheap and convenient and often replace healthier options such as roasted chana, nuts or whole fruit.

6. No Simple Visual Guide: Without a clear visual aid, planning a balanced plate feels confusing, especially for people with limited health literacy.

7. Budget and Access Constraints: Many patients have limited income and limited access to dietitians, so advice must be low-cost and based on locally available foods.

8. Fear of Being Judged: People hesitate to ask questions about food for fear of blame or shame; an open, judgement-free conversation and discussion of diet and medicines with the care team are needed.

5.2 Short-Term Action Plan for Immediate Community Awareness

Immediate steps should focus on spreading the plate-method message through simple visuals and short conversations. This includes displaying and distributing the "Smart Meals" handout in wards and waiting areas, holding small group talks for patients and attendants, encouraging measured portions of rice or roti with dal, vegetables and curd, and promoting simple swaps such as whole fruit instead of juice, water instead of sweetened drinks and roasted chana instead of packaged snacks. Patients should be encouraged to discuss their diet and medicines with their doctor or dietitian.

5.3 Long-Term Action Plan for Sustainable Hospital Nutrition Counselling

For lasting change, diet counselling and plate-method education should become a regular part of diabetes care at the hospital with the help of doctors, nurses and dietitians. Posters in the local language showing familiar meals can be placed in wards and out-patient areas, and attendants and families should be included in the sessions. Periodic awareness camps, follow-up sessions, simple meal charts for different budgets and training of student volunteers and health workers can help sustain healthy habits. Progress should be reviewed regularly and the material updated on the basis of patient responses.

5.4 Community Awareness Programmes Conducted and Observable Outcomes

To address the needs identified in the community, I carried out the following awareness activities through the handout, simple explanations and conversations with patients and attendants in the hospital wards:

1. Plate Method Demonstration: I showed patients and attendants how to divide a plate: half with non-starchy vegetables such as spinach, beans, cabbage, okra, tomato and cucumber; a quarter with protein such as eggs, fish, chicken, tofu, paneer or dal; and a quarter with measured rice, roti or millet, along with water or an unsweetened drink.

- **Outcome:** The plate method gave them a simple, repeatable way to think about a balanced meal instead of a list of banned foods.

2. Carbohydrate and Portion Awareness: I explained the three steps of recognise, measure and pair. Rice, roti, fruit, milk and pulses all contribute carbohydrates; a fist-sized portion of rice or roti is a practical serving; and adding dal, salad or curd slows the rise in blood glucose.

- **Outcome:** Participants were introduced to portion size and food pairing as part of the meal plan.

3. Simple Food and Drink Swaps: I shared low-cost swaps: whole fruit instead of fruit juice, water instead of sweetened drinks, baked, roasted or steamed foods instead of deep-fried snacks, roasted nuts or chana instead of chips or biscuits, and less sugar in tea or coffee. I also explained a flexible day of meals from breakfast to dinner.

- **Outcome:** Small, realistic swaps were presented as easier to sustain than strict food bans.

4. Open Conversation and Question Session: I listened to their questions without blame or shame about food choices and encouraged them to discuss diet and medicines with their care team.

- **Outcome:** A friendly, judgement-free interaction that helps reduce myths and fear about diabetes and food.



Figure 5.1: Balanced Home-Style Indian Meal — Whole-Wheat Rotis, Mixed Vegetables, and Dal Providing Sustained Energy and Glycaemic Control.

CHAPTER 6: RECOMMENDATIONS AND THE CONCLUSION OF THE CSP

6.1 Practical Dietary Recommendations for People with Diabetes

Based on my field interaction, the needs identified and the awareness activities conducted, the following recommendations are made to help people with diabetes build balanced, affordable meals:

1. Promote the Plate Method:

o Display plate-method posters with familiar local meal examples in wards, waiting areas and out-patient areas.

o Encourage families to fill half the plate with non-starchy vegetables and keep rice or roti to a measured quarter.

2. Encourage Healthier Everyday Swaps:

o Promote whole fruit, water or unsweetened drinks, and baked, roasted or steamed snacks instead of juices, sweetened drinks and fried snacks.

o Keep familiar foods and adjust portions and pairings instead of banning foods.

6.2 Healthcare System and Community-Level Recommendations

3. Strengthen Counselling and Care-Team Support:

o Include short diet counselling in routine diabetes care with doctors, nurses and dietitians.

o Encourage patients to discuss diet and medicines with their care team before making major changes.

4. Build Community Awareness and Reduce Myths:

o Conduct periodic awareness sessions for patients and attendants in simple language, with open and judgement-free conversation.

o Involve family members and attendants, since they often choose and prepare the meals.

5. Collaborate and Monitor Progress:

o Partner with hospital staff, local health workers and student volunteers to sustain the awareness programme.

o Review community needs regularly and update the awareness material accordingly.

6.3 Conclusions

The project showed me that diabetes-friendly eating does not mean giving up familiar foods; it means rebalancing them. Through the plate method, awareness of carbohydrate portions and pairing, simple swaps and a flexible day of meals, I presented a practical and low-cost way of building balanced Indian meals for people with diabetes.

Community awareness and open, judgement-free conversation help reduce myths and fear around food and diabetes. Short-term actions such as visual handouts, small talks in the wards and simple swaps can be started immediately, while long-term strategies such as regular counselling, local-language posters and periodic awareness camps are essential for lasting benefits.

Overall, the project helped me spread a practical health message while building my communication, listening and teamwork skills under the guidance of **Ms. T. Navya Chandrika**. Implementation of the recommended actions by the concerned authorities can help patients manage their blood sugar better and improve their quality of life. This report is meant for awareness only and does not replace the advice of a doctor or a qualified dietitian.

6.4 Visual Summary of Sustainable Smart Meal Practices



Figure 6.1: Regular Blood Glucose Monitoring Combined with Healthy Nutrition — Key to Effective Diabetes Management.

Student Self-Evaluation for the Community Service Project

Student Name: **BOMMALI DILEEP**

Registration No: **24KN1A4425**

Period of CSP: **8 Weeks** From: **03/05/2026** To: **27/06/2026**

Date of Evaluation:

Please rate your performance in the following areas:
Rating Scale: Letter grade of CGPA calculation to be provided

1	Oral communication	1	2	3	4	5
2	Written communication	1	2	3	4	5
3	Proactiveness	1	2	3	4	5
4	Interaction ability with community	1	2	3	4	5
5	Positive Attitude	1	2	3	4	5
6	Self-confidence	1	2	3	4	5
7	Ability to learn	1	2	3	4	5
8	Work Plan and organization	1	2	3	4	5
9	Professionalism	1	2	3	4	5
10	Creativity	1	2	3	4	5
11	Quality of work done	1	2	3	4	5
12	Time Management	1	2	3	4	5
13	Understanding the Community	1	2	3	4	5
14	Achievement of Desired Outcomes	1	2	3	4	5
15	**OVERALL PERFORMANCE**	1	2	3	4	5

Date: Signature of the Student

Evaluation by the Person in-charge in the Community / Habitation

Student Name: **BOMMALI DILEEP**

Registration No: **24KN1A4425**

Period of CSP: **8 Weeks** From: **03/05/2026** To: **27/06/2026**

Date of Evaluation:

Name of the Person in-charge: **Dr. Srinivas**

Address with mobile number: **GGH / SMC, Vijayawada**

Please rate the student's performance in the following areas:

Please note that your evaluation shall be done independent of the student's self-evaluation

Rating Scale: 1 is lowest and 5 is highest rank

1	Oral communication	1	2	3	4	5
2	Written communication	1	2	3	4	5
3	Proactiveness	1	2	3	4	5
4	Interaction ability with community	1	2	3	4	5
5	Positive Attitude	1	2	3	4	5
6	Self-confidence	1	2	3	4	5
7	Ability to learn	1	2	3	4	5
8	Work Plan and organization	1	2	3	4	5
9	Professionalism	1	2	3	4	5
10	Creativity	1	2	3	4	5
11	Quality of work done	1	2	3	4	5
12	Time Management	1	2	3	4	5
13	Understanding the Community	1	2	3	4	5
14	Achievement of Desired Outcomes	1	2	3	4	5
15	**OVERALL PERFORMANCE**	1	2	3	4	5

Date:

ASSISTANT PROFESSOR
Department of General Medicine
GGH/SMC
VIJAYAWADA

Signature of the Supervisor

PHOTOS:

Field visit: awareness session on “Smart Meals for People with Diabetes” at Government Hospital, Gunadala, Kanuru, Vijayawada, Andhra Pradesh (Conducted on Tuesday, 16/06/2026 during Week 7).









EVALUATION

Internal Evaluation for the Community Service Project

Objectives:

- To facilitate an understanding of the issues that confront the vulnerable / marginalized sections of society.
- To initiate team processes with the student groups for societal change.
- To provide students with an opportunity to familiarize themselves with the urban / Rural community they live in.
- To enable students to engage in the development of the community.
- To plan activities based on the focused groups.
- To know the ways of transforming society through systematic programme implementation.

Assessment Model:

- There shall only be internal evaluation.
- The Faculty Guide assigned is in charge of the learning activities of the students and for the comprehensive and continuous assessment of the students.
- The assessment is to be conducted with 100 marks.
- The number of credits assigned is 4. Later the marks shall be converted into grades and grade points to include finally in the SGPA and CGPA.
- The weightings shall be:
  - o Activity Log 20 marks
  - o Community Service Project Implementation 30 marks
  - o Mini Project Work 25 marks
  - o Oral Presentation 25 marks
- Activity Log is the record of the day-to-day activities. The Activity Log is assessed on an individual basis, thus allowing for individual members within groups to be assessed this way. The assessment will take into consideration the individual student's involvement in the assigned work.
- While evaluating the student's Activity Log, the following shall be considered -
  - a. The individual student's effort and commitment.
  - b. The originality and quality of the work produced by the individual student.
  - c. The student's integration and co-operation with the work assigned.
  - d. The completeness of the Activity Log.
- The assessment for the Community Service Project implementation shall include the following components and based on Weekly Reports and Outcomes Description (Details of the Community Survey, Problems identified, Community Awareness Programs organized, Suggested Short-Term and Long-Term Action Plan).

MARKS STATEMENT

(To be used by the Examiners)

INTERNAL ASSESSMENT STATEMENT

Student Name	**: BOMMALI DILEEP**
Programme of Study	**: Bachelor of Technology**
Year of Study	**: 2026-2027**
Group	**: CSE – DATA SCIENCE**
Registration No	**: 24KN1A4425**
Name of the College	**: NRI Institute Of Technology**
University	**: JNTUK**

Sl. No	Evaluation Criterion	Maximum Marks	Marks Awarded
1.	Activity Log	20	
2.	Community Service Project Implementation	30	
3.	Mini Project Work	25	
4.	Oral Presentation	25	
	GRAND TOTAL	**100**	

Date: **Signature of the Faculty Guide**

Certified by

Date: **Signature of the Head of the Department/Principal**

Seal:



ANDHRA PRADESH
STATE COUNCIL OF HIGHER EDUCATION

(A Statuory Body of the Government of Andhra Pradesh)

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